



OFFICIAL

Obstetric Cholestasis Guideline

1. Purpose

This guideline provides guidance on the management options of Obstetric Cholestasis in order to minimise its associated maternal and fetal risks.

2. Applicability

This guideline applies to medical and midwifery staff working within the Armadale Health Service maternity unit.

3. Background

Obstetric cholestasis (OC) is a multifactorial condition. The diagnosis of OC should be considered in pregnant women who have itching in skin of normal appearance and raised peak random total bile acid concentration of 19micromol/L or more.

There are no clinical features or laboratory patterns that are unique to OC, as other conditions can cause itching, or raised bile acid concentrations in pregnancy. Around 25% of pregnant women develop itching; the majority of these do not have and do not develop OC

Oestrogen is the most important hormonal precipitant, and OC appears when placental oestrogen synthesis peaks in the third trimester, resolving soon after birth.

4. Risk factors

4.1 Maternal

- Obstetric Cholestasis has a genetic predisposition that influences sensitivity to certain hormonal and environmental factors in the third trimester.
- Occurs in 0.1 – 0.2 % of pregnant women in Australia, North America and 1.2-1.5% of Indian Asian or Pakistani Asian origin.
- Is evenly distributed among primiparous and multiparous women.
- It is increased five-fold in multiple pregnancies.
- OC may reoccur in subsequent pregnancies of those affected.
- OC is more common and may present early in women with chronic hepatitis C infection
- Previous pregnancies complicated by pruritis.
- Past history of Gall stones and/or of pruritis while on OCP.
- Family history of OC and /or Gall stones.

4.2 Diagnosis

- The diagnosis of ICP should be considered in pregnant women who have itching in skin of normal appearance and raised peak random total bile acid concentration of 19micromol/L or more.

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- The diagnosis is more likely if it is confirmed that itching and raised bile acids resolve after birth
- Other causes of itching and liver dysfunction should be excluded

Categorisation of pregnant women with itching of normal skin:

Diagnosis	Clinical features
Gestational pruritis	Itching and peak bile acid concentration <19micromol/L
Mild OC	Itching and raised peak bile acid concentration 19-39micromol/L
Moderate OC	Itching and raised peak bile acid concentration 40-99micromol/L
Severe OC	Itching and raised peak bile acid concentration >100micromol/L

N.B: The upper limit of normal bile acid concentration in pregnancy is 18micromol/L

4.3 Differential diagnosis

Exclude any pre-existing liver disease, alcohol or other drug dependence

- Consider viral hepatitis (especially if jaundice and dark urine present): check viral serology, including hepatitis A, hepatitis B, hepatitis C, cytomegalovirus (CMV) and Epstein-Barr virus (EBV)
- OC is more common and may present early in women with chronic hepatitis C infection.
- Autoimmune liver disease may rarely present in pregnancy, but should be considered, especially if there is a family history of autoimmune disorder (thyroid, rheumatoid, etc).
- Anti-smooth muscle and anti-LKM antibodies (chronic hepatitis), and anti-mitochondrial antibodies (primary biliary cirrhosis), may be checked.
- Pruritic urticarial papules and plaques of pregnancy (PUPP syndrome) and papular dermatitis of pregnancy have accompanying papules and plaques with itching. They may rarely co-exist with OC
- Pre-eclampsia and Acute fatty liver of pregnancy (AFLP) are pregnancy-specific causes of abnormal LFTs and need to be considered in the differential diagnosis.

These diseases can coexist with cholestasis. If a diagnosis of OC is suspected, carry out a structured history and examination, so that other causes of itching and liver dysfunction can be excluded

5. Management

5.1 Investigations

There is no single diagnostic test for OC. **Pregnancy ranges for biochemistry must be used.**

If the itchy skin looks normal, or there is only skin trauma due to scratching, the diagnosis may include gestational pruritus, or OC; measurement of bile acid concentrations and liver function

tests should be undertaken. Raised bile acid concentration of 19micromol/L or more in pregnancy supports a diagnosis of OC.

When clinically indicated, bile acid measurements should be taken at a convenient time, and do not need to be performed fasting.

As prandial readings are higher than fasting, this approach maximises the chance of detecting peak bile acid readings that are of greater clinical importance for preventing adverse pregnancy outcome

All women with itch and an initial raised bile acid level, should have a second bile acid measurement repeated around **one week** later before any diagnostic or care decisions are determined, as it is common for women with bile acid levels over 100micromol/L and 40–100micromol/L to have subsequent bile acid concentrations that are much lower.

The subsequent frequency at which women have biochemical assessment will be determined on an individual basis and according to the impact that the result might have on further care:

- If the woman has mild OC with peak bile acids 19–39micromol/L, they could have weekly testing as they approach 38weeks' gestation in order to inform timing of birth
- If the woman has moderate OC with peak bile acid 40–99micromol/L, especially if they are approaching 35weeks' gestation, weekly testing should be considered, as timing of birth may be influenced if levels rise to 100micromol/L or more.
- If the woman has severe OC with peak bile acid 100micromol/L or more, further routine testing of bile acids might not impact on decision making and therefore may not be routinely required.
- In women with persistent itch of normal skin and normal blood results, an initial diagnosis of gestational pruritus should be considered. Women can go on to develop OC up to 15weeks after a diagnosis of gestational pruritus.
- If itching continues for these women, they should be offered review with repeated liver function tests and bile acid measurement as clinically indicated.
- Coagulation testing is not recommended routinely for women with uncomplicated OC. The small number of abnormal coagulations were in women with alternative diagnoses e.g., acute fatty liver of pregnancy.

5.2 Antepartum management:

- Women with OC to be reviewed by specialist obstetricians.
- Once OC diagnosed, GP Obstetrician or specialist Obstetrician to follow up with patients every two weeks in outpatient clinic.
- Midwifery appointments to continue as per routine schedule.
- Clinicians should be aware that fetal ultrasound and/or cardiotocography (CTG) do not predict or prevent stillbirth in OC. This should be discussed with the women
- OC is not associated with fetal growth restriction, with no difference in birthweight centiles compared with babies born to women without OC, and therefore strategies for antenatal monitoring for placental insufficiency are unlikely to be beneficial in women with isolated OC, however ultrasound (USG) for fetal wellbeing and biometry can be done at least once in our unit
- If USG normal, there is no need to do CTG. In general, the lack of predictability of future fetal wellbeing of a normal CTG is a major limitation of the use of this modality.

- If some abnormality is noted on USG, then CTG to be done according to the need and nature of abnormality.
- No AAU follow up for OC patients unless woman has other comorbidities
- If woman does not come in spontaneous labour, Plan IOL as follows:
- Consider options of planned birth by 40weeks' gestation or ongoing antenatal care in women with mild OC (peak bile acids 19–39micromol/L) and no other risk factors; advise women that the risk of stillbirth is similar to the background risk
- Consider planned birth at 38–39weeks' gestation in women with moderate OC with peak bile acids 40–99micromol/L and no other risk factors; advise them that the overall risk of stillbirth is similar to the background risk until 38–39weeks' gestation
- Consider planned birth at 35–36weeks' gestation in women with severe OC with peak bile acids 100micromol/L or more; advise them that the risk of stillbirth is higher than the background risk
- Advise women that the presence of comorbidities (such as gestational diabetes, pre-eclampsia, multifetal pregnancy) appear to increase the risk of stillbirth and may influence decision making around timing of planned birth

5.3 Fetal risks

- Preterm labour (spontaneous or iatrogenic)
- Meconium-stained liquor
- Still birth risk is 0.13% in women with peak bile acids less than 40micromols/L, which is not higher than background population risk.
- Stillbirth in women with peak bile acids of 40– 99micromol/L was 0.28%. This was not higher than over all background population risk, but did appear to increase at around 38–39weeks' gestation
- For women with peak bile acids 100micromol/L or more, the risk of stillbirth is 3.44%, which is higher than background population rate, and the risk appears to increase from 35–36weeks' gestation.

5.4 Pharmacological management

- Advise women that there are no treatments that improve pregnancy outcome or raised bile acid concentrations and treatments to improve maternal itching are of limited benefit.
- Do not routinely offer Ursodeoxycholic acid for the purpose of reducing adverse perinatal outcomes in women with OC
- Use of UDCA may be considered if OC is diagnosed remote from term to improve maternal symptoms. Start with 250 mg three times a day and up to 750 mg three to four times a day, depending on symptoms. Woman should be informed clearly that there is no foetal benefit and is purely for maternal symptom alleviation.
- Consider topical emollients such as aqueous cream (with or without menthol added) to ameliorate skin symptoms
- Consider antihistamine agents, such as chlorphenamine, particularly at night although the effectiveness of this treatment is uncertain in women with OC
- Consider maternal vitamin K treatment only if there appears to be reduced absorption of dietary fats (e.g., presence of steatorrhoea) and/or evidence of abnormal prothrombin time if coagulation studies are performed.

5.5 Intrapartum management:

- Offer continuous electronic fetal monitoring (CEFM) to women with peak bile acids 100micromol/L or more.
- There is insufficient evidence for or against CEFM in women with peak bile acids below 100micromol/L. A shared decision can be made based on co-morbidities and preferences.
- Advise women that the presence of risk factors (such as gestational diabetes, pre-eclampsia, multifetal pregnancy) appear to increase the risk of adverse perinatal outcomes and that these conditions themselves may necessitate monitoring during birth or in conjunction with OC may influence decision-making around monitoring in labour.
- Advise women that meconium-stained liquor is more common in moderate and severe OC, and that this will influence decision making around CEFM
- Offer women with uncomplicated OC standard analgesia and anaesthesia options for birth
- Active third stage of management

5.6 Postpartum management

- For women who have uncomplicated OC, follow-up should be arranged at least 4weeks after birth to confirm resolution of OC
- A physician referral can be made by the GP if biochemical abnormalities persist to exclude underlying liver abnormalities
- Advise women that OC itself does not influence their choice of contraception or hormone replacement therapy
- For women with OC and previous cholestasis secondary to combined hormonal (oestrogen-containing) contraception, advise them to use progestogen only or non-hormonal
- Advise women with a history of OC that they have an increased chance of recurrence of OC in subsequent pregnancies
- Perform a baseline liver function test and bile acid concentration with booking blood investigations

6. Policy context

The following documents are required to give effect to this policy:

- HDWA
 - [Clinical Handover Policy](#)
- EMHS
 - [Fetal Surveillance and Cardiotocography \(CTG\) Monitoring Policy](#)

7. Supporting information

The following documents support the implementation of this policy:

- [Birth – Third Stage of Labour – Active Management Guideline](#)

8. Compliance, monitoring and evaluation

Compliance with this policy will be monitored via:

- Monitoring of reported clinical incident data via Datix CIMS.

Specific findings are to be reported at the departmental meeting for review and identification of any opportunities for improvement.

9. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.11 Clinical assessment
- Clinicians comprehensively assess the conditions and risks identified through the screening process
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:
 - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers and families
 - c. Clinicians who are involved in the clinical handover

10. References

1. Royal College of Obstetricians and Gynaecologists (RCOG). Intrahepatic Cholestasis of pregnancy. RCOG Guideline No. 43; September 2022.
2. King Edward Memorial Hospital. Cholestasis in pregnancy. Obstetrics & Gynaecology Clinical Practice Guideline; June 2019
3. Kondrackiene J, Kupcinskas L. Intrahepatic cholestasis of pregnancy - current achievements and unsolved problems. World Journal of Gastroenterology. 2008;14(38):5781-88.
4. Mays JK. The active management of intrahepatic cholestasis of pregnancy. Current Opinion in Obstetrics and Gynecology. 2010;22:100-3.
5. Greenes V, Williamson C. Intrahepatic cholestasis of pregnancy. World Journal of Gastroenterology. 2009;15(17):2049-66.
6. Lee RH, Kwok KM, Ingles S, et al. Pregnancy Outcomes during an Era of Aggressive Management for Intrahepatic Cholestasis of Pregnancy. American Journal of Perinatology. 2008;25:341-45.
7. Nichols AA. Cholestasis of Pregnancy. A review of the evidence. Journal of Perinatology and Neonatal Nursing. 2005;19(3):217-25.
8. Saleh MM, Abdo KR. Intrahepatic Cholestasis of Pregnancy: Review of the Literature and Evaluation of Current Evidence. Journal of Women's Health. 2007;16(6):833-41.
9. Pathak B, Sheibani L, Lee RH. Cholestasis of Pregnancy. Obstetrics and Gynecology Clinics of North America. 2010;37:269-82.
10. Glantz A, Marschall H-U, Lammert F, Matteson L-A. Intrahepatic cholestasis of pregnancy: a randomized controlled trial comparing dexamethasone and ursodeoxycholic acid. Hepatology 2005; 42: 1399-1405.

Recognition to SA Maternal & Neonatal Clinical Network, South Australian Perinatal Practice Guidelines workgroup for contribution to this guideline.

11. Document contact

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12. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V5	20 02 2026	20 02 2029	Full review, minor grammatical errors only amended.

13. Authorisation

This guideline has been authorised and issued by:

Authorised by	Dr Tania Strickland - Director Clinical Services
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